Dermatology 23S Module 2

Go to Week 3 Academic Forum

Week 3 Scenario 1

Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 19 November 2023, 3:56 PM

Hi Group,

Welcome to week three, here is scenario one.

You are asked to see this 85-year-old man who is a residential home resident. He has a background history of vascular dementia but has generally good functional abilities. He currently takes memantine and donepezil together with amlodipine, aspirin and simvastatin. The staff at the home are concerned about this ulcer on the left side of the nose. The nurses report that it has grown over the last few months and the patient constantly picks at the lesion.

Please answer all the questions below:
1. What is the likely diagnosis?
2. Which important differential must you consider in this patient scenario?
3. What is the most appropriate management step in this patient?
4. What histological feature would support your diagnosis?

126 words

Reply

Re: Week 3 Scenario 1
by Pooran Outar - Monday, 20 November 2023, 12:09 AM

Considering the patient's age, gender, ethnicity, gradual progression over several months, and the distinct features of this lesion (a shiny or pearly nodule with a smooth surface, accompanied by central depression or ulceration resulting in rolled edges), my diagnosis is a **Nodular Basal Cell Carcinoma** (Oakley, 2015).

In this case it would be wise to consider the following **differentials** when diagnosing BCC (Bader, 2022); (Oakley, 2015):

- Squamous Cell Carcinoma in Situ (Bowen's Disease) - Well-defined, red to brownish or tan-hued, scaly patch or plaque; may exhibit asymmetry with a slightly raised irregular border.
- Keratoacanthoma - Rapidly growing, dome-shaped nodule that resembles a crater.
- Sebaceous Hyperplasia - Typically presents as small, yellowish bumps on the face.
- Molluscum Contagiosum - A viral infection that causes small, flesh-colored or pearly bumps.
- Trichoblastoma - A benign tumor originating from hair follicle cells; usually presents as a well-circumscribed nodule.
- Trichoepithelioma - Benign tumor derived from hair follicle cells; appears as a small, skin-colored, or slightly pigmented nodule.

- Seborrheic Keratosis - Often have a waxy, scaly, or rough texture with a stuck on appearance; color can vary but is often tan, brown, or black.
- Dermatofibroma - Commonly appears on the lower extremities; typically brownish and may dimple when pinched.

What other differentials we can consider?

Taking into consideration the patient's age, location of the BCC, possibility of scarring, patient choice and overall health of the patient , I would recommend the following **management**:

- No treatment recommended due to slow growth and low likelihood of recovery given the patient's age (Foundation B.S., n.d.).

Should the patient choose to undergo treatment then:
- The patient should be managed by MDT (Newlands et al., 2016)
- Conciser Topical 5% Imiquimod (Oakley, 2015).
- Excision with a 0.5 cm clinical margin consiering the location of the primary lesion does not allow for a 2 to 3 cm border ressection; MoH Surgery is preffered for infiltrative of recurrent BCC at difficult to manage sites (Newlands et al., 2016).

What other treatment options are available for BCC?

The following **histopathological findings** may help to support this diagnosis (Tallon, 2016):

- Basal cell carcinoma stains positively with cytokeratin.
- Basaloid epithelium typically forms a palisade.
- A cleft forms from the adjacent tumor stroma.
- Nuclei become crowded centrally, with scattered mitotic figures and evident necrotic bodies.
- Presence of a mucinous stroma is a distinguishing feature from other basaloid cutaneous tumors.
- Some tumors may exhibit foci of regression, presenting as areas of eosinophilic stroma with a lack of basaloid nests.
- **In nodular BCC**, the tumor forms solid nodules that may extend into subcutaneous tissues. Cartilaginous invasion is unusual.

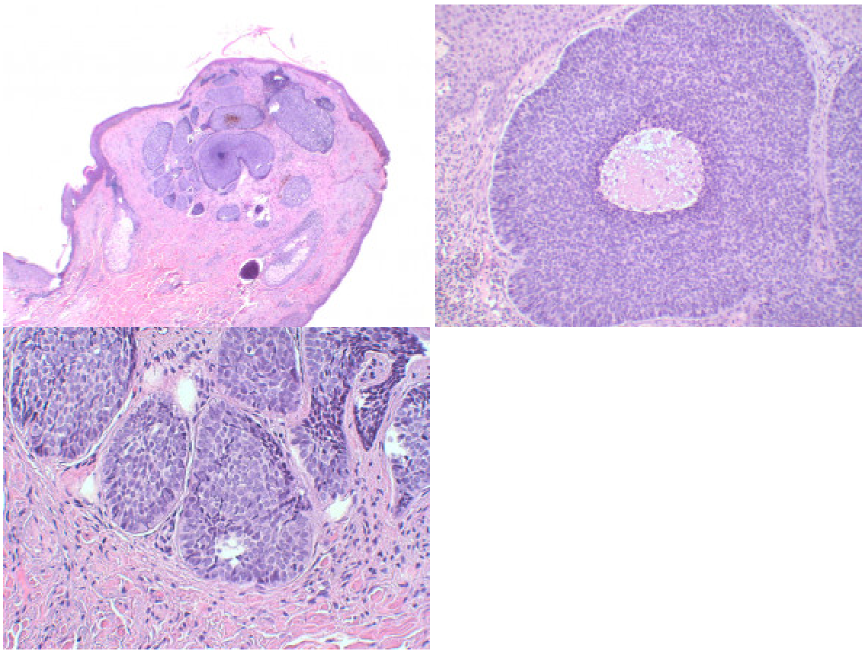

Image Source (Tallon, 2016)

References:

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Newlands, C., Currie, R., Memon, A., Whitaker, S. and Woolford, T. (2016). Non-melanoma skin cancer: United Kingdom National Multidisciplinary Guidelines. *The Journal of Laryngology & Otology*, [online] 130(S2), pp.S125–S132. doi:https://doi.org/10.1017/s0022215116000554.

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519 words

Reply

Re: Week 3 Scenario 1
by Adam Tollitt - Monday, 20 November 2023, 10:20 AM

Hi Pooran,

Thank you for your comprehensive post. I agree with your diagnosis of nodular BCC for this case.

To expand further on the management of this case, I agree that there are several potential management options for this patient and a holistic approach is required when choosing the most appropriate management.

The gold standard treatment option for this lesion, particularly given its higher risk location on the patient’s nose, would be Mohs micrographically controlled surgery. This approach would achieve a high cure rate due to intraoperative histological analysis allowing for complete excision, whilst maximally preserving healthy tissue to leave a better cosmetic outcome (Oakley, 2015).

In this case, the patient is elderly with vascular dementia. I agree with Pooran that an MDT approach is required to consider the most appropriate treatment option. It would also be important to ascertain further information about the patient’ functional status and whether he himself is able to make an informed decision about treatment options or whether the views of any relatives/NOK are required. In the UK, sometimes people with dementia will have a Lasting Power of Attorney for Health and Welfare which is a legal status granted to an individual (often close relative/friend) to allow them to make decisions on the patient’s behalf if they do not have capacity to decide.

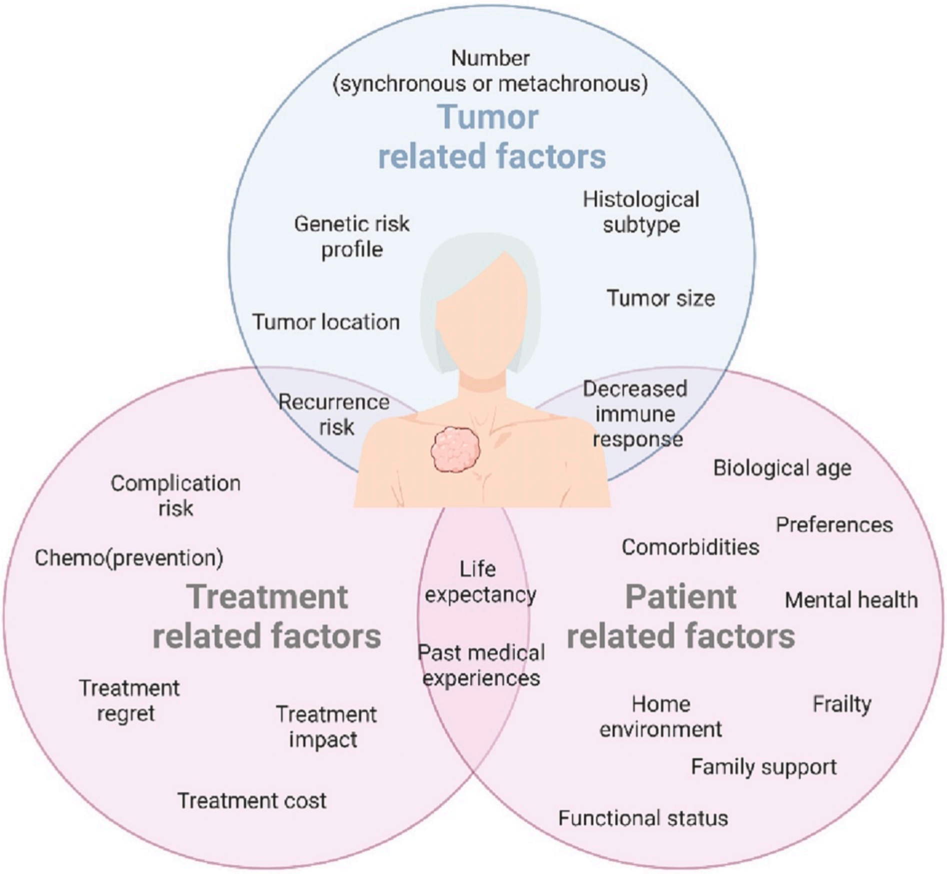

Figure 1 - some of the important factors that need to be considered when making decisions about management of BCCs in elderly patients specifically (Laura Van Coile et al., 2023)

Despite being the gold standard treatment option and being a safe procedure in elderly patients, Mohs micrographic surgery is unlikely to be a suitable for this patient due to its longer procedural time (Linos et al., 2013).

Observation may be another management option and is often considered in elderly patients with a shorter life expectancy and multiple co-morbidities making other treatment options unsuitable (Wiznia and Federman, 2016). As this patient is frequently irritated by the lesion (noted to be constantly picking at it) I feel some form of management intervention is indicated.

I have found limited evidence to conclusively recommend one treatment option over another in the elderly/dementia patient cohort, however the evidence that I have read suggests topical medical treatments, cryotherapy, conventional surgical excision or electrodessication & curettage as potentially suitable options (Wiznia and Federman, 2016; Laura Van Coile et al., 2023).

Modality	Advantages	Disadvantages
Mohs micrographic surgery	Complete margin analysis Well tolerated by elderly Gold standard treatment	Cost Longer procedure (stages)
Conventional surgical excision	Well tolerated by elderly	Cost Lack of complete margin analysis
Electrodessication and curettage	Shorter procedure Does not require return visit Patients can avoid surgery	Lack of histologic confirmation of malignancy removal Not appropriate for lesions with extension into deep dermis
Cryosurgery	Patients can avoid surgery	Higher recurrence rates than surgery Lack of histologic confirmation of malignancy removal Recurrent carcinoma could become extensive (can be obscured by fibrous scar tissue) Hypertrophic scarring Postinflammatory pigment changes
Imiquimod	Patient self-administration Excellent cosmetic results	Local skin reactions Lack of histologic confirmation of malignancy removal Cost
Photodynamic therapy	Excellent cosmetic outcome	Higher recurrence rates than with surgery Lack of histologic confirmation of malignancy removal
5-FU	Patient self-administration	Higher recurrence rates than with surgery
Observation	Patients can avoid surgery Cost	No standard as to length of time for which it is appropriate to monitor patients clinically More dangerous neoplasm may be missed (such as Merkel cell carcinoma or amelanotic melanoma)

Figure 2. – summary of the advantages and disadvantages of basal cell carcinoma treatment options in the elderly (Wiznia and Federman, 2016).

Question for the group:

Has anyone been involved in MDT decision making for any older patients with non-melanocytic skin cancers?

What important patient, lesion and treatment factors were considered in the MDT decision making process?

References

Laura Van Coile, Verhaeghe, E., Ongenae, K., Destrooper, L., Mohamadi, Z., Liève Brochez and Hoorens, I. (2023). The therapeutic dilemma of basal cell carcinoma in older adults: A review of the current literature. *Journal of Geriatric Oncology*, 14(3), pp.101475–101475. doi:https://doi.org/10.1016/j.jgo.2023.101475.

Linos, E., Parvataneni, R., Stuart, S.E., Boscardin, W.J., Landefeld, C.S. and Chren, M.-M. (2013). Treatment of Nonfatal Conditions at the End of Life. *JAMA Internal Medicine*, 173(11), p.1006. doi:https://doi.org/10.1001/jamainternmed.2013.639.

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562 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:12 PM

Thank you Adam, very good post.

6 words

Reply

Re: Week 3 Scenario 1
by Sophie Holloran - Tuesday, 21 November 2023, 9:05 PM

Hi Adam and Pooran,

Thank you both for your posts. I will definitely read the paper you have referenced by Laura Van Coile, as this is something I am very interested in.

I agree with you both that this appears to be a BCC.

In answer to your question, I have been involved in decision making with similar patients. I think the chart that you have posted is a comprehensive summary of what should be considered.
From my experience, I think that another important factor is how much the lesion is bothering the patient - if it is bleeding, or really itchy or painful, then that might be a reason to consider surgical excision in an otherwise very frail patient.
In a frail patient who has capacity and is able to understand the risks of leaving the lesion untreated and the risks of surgery, I think clear communication and patient choice is the most important. A patient always has the option to change their mind, and if appropriate maybe a follow up appointment or telephone call could be an interim measure.
If a patient does not have capacity, then it can be much more difficult. In that scenario I think taking time to talk to relatives and carers (LPA etc) is very important.

213 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:11 PM

Thank you Poorhan, this is indeed a Basal Cell Carcinoma, well done very good post.

15 words

Reply

Re: Week 3 Scenario 1
by Jillian Simpson - Monday, 20 November 2023, 11:23 AM

Diagnosis
I think the most likely diagnosis here is a basal cell carcinoma (BCC). Features that support this diagnosis are his age, potentially his skin type (he looks fair skinned with blue eyes), and the area of skin affected. Sun-exposed skin in the 'masked area' of the face is most frequently affected (Morris-Jones, 2019).

Other information that would support this diagnosis: it is slow growing and often bleeds. The case references him picking the lesion, we need to be mindful of his vascular dementia so we may not know for sure if the bleeding is a direct cause of picking or more spontaneous in nature.

From looking closely at the picture, the surface epidermis looks like it has broken down to cause an ulcer, with the residual edges of the nodule forming the characteristic 'rolled edge', so it is most likely to be a nodular

BCC. These are the most common types of facial BCC's (Oakley, 2015).

Differentials
My main differential would be a squamous cell carcinoma (SCC), as these are often nodular and can ulcerate (Oakley, 2015). These typically grow faster than BCC's. Another differential would be a keratoacanthoma, thought to be a variant of SCC these grow rapidly, have a pinkish symmetrical cup shaped nodule, appear on sun damaged skin, have smooth rolled edges and a central keratin plug (Burge, Wallis, Martin, 2016).

Management
Nodular BCC's on the face are usually treated by Mohs' micrographic surgery as this is a high risk site and this type of surgery has a very high cure rate (Oakley, 2015). The issue we have in this case is this gentleman's vascular dementia and whether he could endure the procedure. We also need to know if this is a primary lesion or if it is a reoccurrence. I think in practice, his case would go to a multidisciplinary team meeting to decide on the most appropriate management plan for him.

Histological features
The histological features of a BCC show collections of basaloid tumour cells, the patterns of which determine the sub type e.g. nodular, superficial, morphoeic, cystic etc (Morris-Jones, 2019).

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Morris-Jones, R. (2019). ABC of Dermatology. Seventh Edition. Wiley Blackwell. P196-197
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Burge, S.M., Wallis, D. and Matin, R. (2016). Oxford handbook of medical dermatology. Oxford: Oxford University Press. p350-351.

410 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:12 PM

Thank you Jillian, good post this is indeed basal cell carcinoma.

11 words

Reply

Re: Week 3 Scenario 1
by Siddharth Saluja - Monday, 20 November 2023, 12:34 PM

1) Given the presentation the most likely diagnosis is Nodular Basal cell carcinoma
2) Most important differentials to consider: (1)

The term "Bowen's Disease" refers to a well-defined, scaly, reddish-brown or tan-colored area or plaque that may have an uneven border and asymmetry. (1)
Keratoacanthoma: A dome-shaped nodule that grows quickly and resembles a crater.(2)
Sebaceous Hyperplasia: This condition usually manifests as little, yellowish pimples on the visage.
Molluscum Contagiosum: A viral infection that results in tiny, pearly or flesh-colored pimples.
Trichoblastoma: A benign tumor that develops from cells in hair follicles; typically appears as a well-defined nodule.(2)
Trichoepithelioma is a benign tumour that develops from hair follicle cells. It manifests as a tiny lump that is skin-colored or faintly pigmented.
Seborrheic Keratosis: May be waxy, scaly, or rough in texture, giving the appearance of being glued on; color may vary, but is typically tan, brown, or black.
Dermatofibroma: Usually brownish in color, this condition primarily affects the lower extremities.(2)
3)The management of basal cell carcinoma depends on several factors, including the size, location, and characteristics of the tumor. Here are some common management approaches. (3)

Surgical Excision: This is the most common treatment method for basal cell carcinoma. It involves removing the tumor along with a margin of healthy tissue to ensure complete removal. This can be done through various techniques like standard excision, Mohs micrographic surgery, or electrosurgery.
Cryotherapy: In some cases, small and superficial basal cell carcinomas can be treated with cryotherapy. This involves freezing the tumor with liquid nitrogen, causing it to slough off. (3)
Curettage and Electrodessication: This procedure involves scraping the tumor with a spoon-shaped instrument called a curette and then using an electric current to destroy any remaining cancer cells.
Topical Medications: Certain topical medications, such as imiquimod or 5-fluorouracil, can be used to treat superficial or early-stage basal cell carcinomas. These medications work by stimulating the body's immune response to target and eliminate cancer cells.(3)
Radiation Therapy: Radiation therapy may be considered for larger, difficult-to-treat tumors or for individuals who are not suitable candidates for surgery. It involves using high-energy X-rays or other forms of radiation to kill cancer cells.
4) The most common histological features supporting the diagnosis include large nests or islands of malignant basaloid cells with haphazardly arranged cells in the centre and peripheral palisading, tumour-stroma clefting, mucoid/myxoid stroma with spindle cells, and amyloid deposits present under the microscope in nodular BCC. The tumor stroma may also occasionally have a collagenous, keloidal-type aspect. The dermis is deeply penetrated by malignant tumor nests, and the core region contains apoptotic cells. The secondary findings that describe nodular BCC reveal many subtypes: adenoid (with cribriform arrangement of tumor nests), cystic/nodulocystic (cystic degeneration), and keratotic (with mature keratin deposits found central in the tumor islands. (4)

Figure 1 BASALOID EPITHELIAL TUMOUR ARISING FROM THE EPIDERMIS (3)

FIGURE 2 :

The basaloid epithelium typically forms a palisade with a cleft forming from the adjacenttumour stroma (3)

FIGURE 3: Centrally the nuclei become crowded with scattered mitotic figures and necrotic bodies evident(4)

References:
1) Niculet, E., Craescu, M., Rebegea, L., Bobeica, C., Nastase, F., Lupasteanu, G., Stan, D., Chioncel, V., Anghel, L., Lungu, M. and Tatu, A. (2021). Basal cell carcinoma: Comprehensive clinical and histopathological aspects, novel imaging tools and therapeutic approaches (Review). Experimental and Therapeutic Medicine, 23(1). doi:https://doi.org/10.3892/etm.2021.10982.
2)dermnetnz.org. (n.d.). Basal cell carcinoma pathology | DermNet NZ. [online] Available at: https://dermnetnz.org/topics/basal-cell-carcinoma-pathology.
3)Wu, A., Sun, M.T., Huilgol, S.C., Madge, S. and Selva, D. (2014). Histological subtypes of periocular basal cell carcinoma. Clinical & Experimental Ophthalmology, 42(7), pp.603–607. doi:https://doi.org/10.1111/ceo.12298.
4)Andréas Astier. (n.d.). Main Type of Skin Cancer — Their Differences, Histology and Prevention. [online] Available at: https://www.andreasastier.com/blog/main-type-of-skin-cancer-their-differences-histology-and-prevention.

613 words

Reply

Re: Week 3 Scenario 1
by Xiao Liey Ding - Monday, 20 November 2023, 12:47 PM

Question 1

Diagnosis: Nodular basal cell carcinoma (BCC)

Supporting factors:

· patient's age

· the location of the lesion on sun-exposed skin in the 'masked area' of the face

· Skin color lesion with central depression and rolled edge suggestive of BCC (Oakley, 2015)

Question 2

Differential diagnoses:

· squamous cell carcinoma (SCC)

· keratoacanthoma, a frequently encountered, swiftly progressing, and locally invasive skin tumor. Keratoacanthomas (KAs) have the potential for spontaneous regression accompanied by scarring. However, their clinical appearance may closely resemble that of well-differentiated squamous cell carcinoma (SCC), and their course can be unpredictable

· Molluscum contagiosum, skin infection caused by a pox virus. A firm, smooth, umbilicated papules, usually 2-5 mm in diameter (Tidy, 2021)

Question 3

Management:

The management of a basal cell carcinoma (BCC) is contingent on factors such as its type, size, location, the quantity requiring treatment, patient-related considerations, and the preferences or expertise of the physician. Surgical intervention is the most common approach for the majority of BCCs (Oakley,2015). The preferred treatment for this lesion, especially given its elevated risk location on the patient's nose, is Mohs micrographically controlled surgery. This method ensures a high cure rate by enabling intraoperative histological analysis for complete excision while preserving healthy tissue to optimize cosmetic outcomes.

Considering the patient's elderly age and the presence of vascular dementia on Memantine and Donepezil, decision making capacity assessment is first done to determine whether he can independently make informed decisions about treatment options.

Other treatment options that can be explored with patient are:

· superficial skin surgery

· cryotherapy (best avoided for lesions on head and neck, and distal knees hence less favourable option for this patient)

· photodynamic therapy

· Imiquimod cream

· Fluorouracil cream

· Radiotherapy is a less frequent treatment approach, being used for unresectable tumors or in cases where surgery is not recommended (Niculet *et al*, 2022)

Question 4

Histological Features:

Microscopically, nodular basal cell carcinoma (BCC) is identified by the presence of extensive clusters or islands of malignant basaloid cells. These cells exhibit a central, disordered arrangement and are surrounded by peripheral palisading.

Additional features include tumor-stroma clefting, mucoid/myxoid stroma containing spindle cells, and the presence or absence of amyloid deposits. Occasionally, the tumor stroma may display a collagenous, keloidal-type appearance. The malignant nests penetrate deep into the dermis, and centrally located apoptotic cells can be observed. Nodular BCC manifests in various subtypes distinguished by secondary characteristics: keratotic (featuring mature keratin deposits at the center of tumor islands), cystic/nodulocystic (experiencing cystic degeneration), and adenoid (exhibiting a cribriform arrangement of tumor nests) (Niculet *et al*, 2022).

References:

Oakley, A. (2015). Basal cell carcinoma | DermNet NZ. [online] dermnetnz.org. Available at: https://dermnetz.org/topics/basal-cell-carcinoma. [Accessed 20/11/2023].

Tidy, C. (2021). Basal cell carcinoma| Patient. [online]patient.info. Available at: https://patient.info/doctor/basal-cell-carcinoma [Accessed 20/11/2023]

Niculet, E. *et al* (2022). Basal Cell Carcinoma: Comprehensive Clinical and Histopathological Aspects, Novel Imaging Tools and Therapeutic Approaches (Review). *Exp Ther Med 23(1):60*. Doi: 10.3892/etm.2021.10982

487 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:13 PM

Thank you Siddharth, very good post this is basal cell carcinoma.

11 words

Reply

Re: Week 3 Scenario 1
by Siddharth Saluja - Wednesday, 22 November 2023, 12:47 PM

pleasure thanks

2 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Monday, 20 November 2023, 3:31 PM

Hi group,
What a fantastic start to the week you are all on the right track !. I shall comment on individual posts and post my thoughts tomorrow when a few other have had a chance to respond. In the meantime I have posted follow on questions.

47 words

Reply

Re: Week 3 Scenario 1
by Arwaf Benrawwaf - Tuesday, 21 November 2023, 4:47 AM

1. The likely diagnosis for the ulcer on the left side of the nose in this 85-year-old man with a history of vascular dementia and constant picking at the lesion is a non-healing or chronic ulcer, possibly a basal cell carcinoma (BCC) or squamous cell carcinoma (SCC). There are different types of BCC: (Oakley, A.2015). (*UpToDate,* 2022)

Nodular basal cell carcinoma: known as nodulocystic carcinoma, and it's the most common facial basal cell carcinoma type. characterized by a raised, firm, and shiny nodule on the skin. It may have small blood vessels on the surface and can sometimes ulcerate. (Oakley, A.2015). (*UpToDate,* 2022)

(Oakley, A.2015).

Superficial basal cell carcinoma: This type of BCC appears as a red, scaly patch on the skin and is more commonly found on the trunk and extremities. It tends to grow slowly and is less likely to invade surrounding tissues. (Oakley, A.2015). (*UpToDate,* 2022)

(Oakley, A.2015).

Morphoeic BCC: s a less common but more aggressive type of BCC, often presenting as a white or yellowish scar-like lesion with poorly defined borders. It can be difficult to detect and may invade surrounding structures. (Oakley, A.2015). (*UpToDate,* 2022)

(Oakley, A.2015).

Basosquamous carcinoma: rare and aggressive type of skin cancer that exhibits characteristics of both basal cell carcinoma (BCC) and squamous cell carcinoma (SCC). present as a raised, firm, and shiny nodule similar to BCC, but it may also have features of SCC such as ulceration and rapid growth.(Oakley, A.2015). (*UpToDate,* 2022)

(Oakley, A.2015).

2. An important differential diagnosis to consider in this patient scenario would be a benign lesion such as a non-healing pressure ulcer, actinic keratosis, or even a non-melanoma skin cancer like BCC or SCC. (*UpToDate,* 2022)

3. The most appropriate management step in this patient would be to refer him to a dermatologist for further evaluation and potential biopsy of the lesion. Given his history of vascular dementia and constant picking at the lesion, it may also be important to involve a multidisciplinary team including geriatricians, nurses, and caregivers to address his behavioral issues and provide appropriate wound care. (McDaniel, B 2022).

4. Histological features that would support a diagnosis of BCC include peripheral palisading of cells, retraction artifacts, and clefting at the tumor-stroma interface. For SCC, histological features may include keratin pearls, intercellular bridges, and atypical squamous cells invading the dermis. A biopsy would be necessary to confirm the diagnosis and guide further management. (*UpToDate,* 2022) (McDaniel, B 2022).

Question: How can we educate and support the patient in preventing further sun damage and promoting skin health given the history of chronic sun exposure and the development of a potentially cancerous skin lesion?

References:

-Oakley, A. (2015). *Basal cell carcinoma | DermNet NZ*. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/basal-cell-carcinoma.

-www.uptodate.com. (n.d.). *UpToDate*. [online] Available at: https://www.uptodate.com/contents/epidemiology-pathogenesis-clinical-features-and-diagnosis-of-basal-cell-carcinoma?search=basal%20cell%20carcinoma%20&source=search_result&selectedTitle=1~150&usage_type=default&disp [Accessed 21 Nov. 2023].

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504 words

Reply

Re: Week 3 Scenario 1
by Adam Tollitt - Tuesday, 21 November 2023, 10:23 AM

Hi Arwaf,

I would like to pick up on your question regarding patient education/support in relation to sun protections measures to help to reduce risk of sun exposure related skin pathologies/cancer.

I suspect that there is no 'one size fits all' answer to this question. A recent literature review of the effectiveness of various interventions to increase sunscreen use in adults found that no one strategy was found to be clearly effective (Allen and Damian, 2021).

Patient education/support with regards to sun protection measures needs to be considered at an individual patient level with a focus on individual clinicians being knowledgeable about the potential harmful effects of frequent, long standing sun exposure; particularly in high-risk patient populations/groups (e.g. Fitzpatrick skin types 1 & 2 and those living in countries with high UV index). It is important for clinicians to highlight these risks to patients and provide sun protection education both opportunistically and when patients present with skin pathologies linked to sun exposure damage. A randomised follow up study in a Swedish primary healthcare setting of patients given different forms of individualised sun protection advice (including by letter and by individualised verbal advice from a GP) found that there were sustained statistically significant declines in sun exposure behaviour patterns for all the forms of individualised sun protection advice provided (Hedevik et al., 2019).

The other consideration is public health promotion. Perhaps the most famous example of this is Australia's 'Slip! Slop! Slap' campaign, first promoted in the summer of 1980-81 (Cancer Council NSW, 2019). This campaign led to a landmark reduction in melanomatous skin cancers in younger age groups. However Australia still has one of the highest rates of skin cancer globally and the lack of a follow up national skin cancer prevention mass-media public health campaign for over a decade in Australia emphasises the importance of sustained public health education/interventions (Walker et al., 2022).

References

Allen, N. and Damian, D.L. (2021). Interventions to Increase Sunscreen Use in Adults: A Review of the Literature. *Health Education & Behavior*, 49(3), p.109019812110465. doi:https://doi.org/10.1177/10901981211046534.

Cancer Council NSW. (2019). *Slip, Slop, Slap, Seek, Slide*. [online] Available at: https://www.cancercouncil.com.au/cancer-prevention/sun-protection/preventing-skin-cancer/slip-slop-slap-seek-slide/. (accessed 21/11/2023).

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Walker, H., Maitland, C., Tabbakh, T., Preston, P., Wakefield, M. and Sinclair, C. (2022). Forty years of Slip! Slop! Slap! A call to action on skin cancer prevention for Australia. *Public Health Research & Practice*, 32(1). doi:https://doi.org/10.17061/phrp31452117.

429 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:18 PM

Thank you Adam, this is a very useful contribution to our discussion, Fitzpartick skin type 1&2 + high UV index leads to increased risk, patient education from an early age and sustained public health campaigns are extremely important.

38 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:14 PM

Thank you Arwaf, good post and very good pictures.

9 words

Reply

Re: Week 3 Scenario 1
by Rahima Abdulaziz Ahmed - Tuesday, 21 November 2023, 9:36 AM

Q1;
Nodular basal cell carcinoma ; given the age of the patient , area of the ulcer, skin tone , the translucency, erosion and ulceration that's central, slow growing, rolled edges and the telangiectasia over the surface . (Bader, 2022)
Q2;
Differential diagnosis in this patient scenario; (Bader, 2022); (Oakley, 2015):
1.Actinic keratoses; precancerous lesion due to sun exposure ;may appear as a rough , scaly patch and could resemble BCC.
2.Squamous cell carcinoma; another type of skin cancer that may present as an ulcerated lesion , more aggressive than BCC
3.Cellulitis can lead to ulceration, especially if the lesion has been picked at.
4.Trauma related ulcer ; constant picking may lead persistent ulceration
5.Melanoma ;can share features like asymmetry and irregular borders ; dermoscopy helps differentiate.
6.Psoriasis plaque; resembles a raised , red plaque but is often symmetric and lacks the translucency of BCC.
Q3;
Appropriate management for this patient ;
-Involves multidisciplinary team as discussed by colleagues earlier , involving a dermatologist , geriatrician, neurologist ,nurses and pharmacists.
-Behavioral intervention ;collaborate with the patients caregivers to implement strategies to reduce picking behavior. This may include distraction techniques , use of gloves or other interventions to protect the lesion. specific management as discussed by Adam ,in addition to scheduling regular follow-up of the patient.
Q4;
Histological features to support the diagnosis;
1.Nodular growth pattern ; characterized by well defined nodules or nests of tumor cells in the dermis.
2.Peripheral Palisading; tumor cells arranged in a palisading pattern at the periphery of tumor nodules.
3.Fibroplasia; presence of fibrous stroma around the tumor nodules.
4.Basophilic cells; tumor cells with basophilic (blue staining) cytoplasm.
5.Clefting; intercellular clefts or spaces between the tumor nodules.
6.Reversed nuclear polarity; nuclei located at the base of the cells, opposite to the normal orientation, within the tumor nodules.
7.increased mitotic activity ;elevated mitotic figures, indicating increased cell division within the tumor nodules.
8.Morpheaform areas; some nodular BCCs may exhibit morphea form (sclerosing) areas with densely packed tumor cells.
9.Infiltrative Borders; infiltration of tumor cells into the surrounding stroma with irregular borders.

References;
Badar, S. (2022) Basal Cell Carcinoma *Medscape* (online) Available at :https://emedicine.medscape.com/article/276624-overview (Accessed on 21/11/23)
Kalmykova, A. and McKee, H. (2023) Basal cell carcinoma *PathologyOutlines.Com* (online) Available at: https://www.pathologyoutlines.com/topic/skintumornonmelanocyticbcc.html (Accessed on 21/11/23)
Oakley ,A .(2015) Basal Cell Carcinoma *DermNet* (online) Available at https://dermnetnz.org/topics/basal-cell-carcinoma (accessed on 21/11/23)

391 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Tuesday, 21 November 2023, 2:25 PM

Hi group, as you have all correctly stated this is a case of Basal cell carcinoma the differential must you consider in this patient scenario is Trigeminal trophic syndrome, a rare condition resulting from self-manipulation of the skin after a peripheral or central injury to the trigeminal system. The syndrome consists of a triad of anesthesia, paresthesia, and a secondary persistent or recurrent facial ulceration frequently of the nasal ala, it usually arises in elderly women and is attributable to damage to the trigeminal nerve.

The most appropriate management step is to arrange for a biopsy. And the histological feature that would support your diagnosis is the presence of Palisading basaloid cells

Basal cell carcinoma (BCC) is the commonest skin cancer and the commonest cancer in the world. The tumour is derived from the basal layer and has an excellent prognosis.

Approximately 85% of BCCs occur on the face. They are slow-growing tumours, enlarging by 0.5 cm over 1-2 years. There are 5 main clinicopathological types: nodular; micronodular; infiltrative; morphoeic; superficial.

The patched/hedgehog intracellular signaling pathway plays a role in both sporadic BCCs and naevoid BCC syndrome (Gorlin syndrome). This pathway influences differentiation of various tissues during foetal development. After embryogenesis, it continues to function in regulation of cell growth and differentiation. Loss of inhibition of this pathway is associated with human malignancy, including BCC. However, in practice PTCH testing is rarely undertaken.

Histologically, there are atypical basaloid nests in the dermis, some connected to the epidermis with palisading in a fibromyxoid stroma.

Sunlight, particularly with prolonged exposure, is the most frequent association with BCC. A latency period of 20-50 years is typical between UV (ultraviolet) exposure and the emergence of BCC. Surgical management is still the gold standard treatment.

290 words

Reply

Re: Week 3 Scenario 1
by Sandeep Kumar - Wednesday, 22 November 2023, 2:56 PM

Thank you for this info.
Trigeminal trophic syndrome as you stated is very rare and I have never heard of this before.
I presume it is more prevalent in patients post stroke where trigeminal nerve is effected in the CVA.
I presume biopsy in that case would be normal as opposed to BCC which will show the mentioned features.

Reference:
Rybak-D Obyrn, J. et al. (2023) ‘Trigeminal trophic syndrome associated with neuralgia after ischemic stroke-a case report’, Clinical case reports, 11(8), p. e7711. doi:10.1002/ccr3.7711.

84 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:19 PM

Hi Sandeep, thank you for your contribution.
Trigeminal trophic syndrome – Trigeminal trophic syndrome happens as the result of trigeminal nerve or sensory nuclei damage. Altered sensation, most commonly around the nose and nasal alar areas, leads to repetitive, self-induced trauma causing ulceration and disfigurement. It less commonly affects other areas on the head or neck. The most frequent causes are therapeutic trigeminal nerve ablation and ischemic medullary or pontine stroke; others include craniofacial surgery, trauma, and herpes zoster infection.

Please have a look at the following papers

https://journals.sagepub.com/doi/10.1177/0145561319850818

https://pubmed.ncbi.nlm.nih.gov/24470665/

https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4976434/

90 words

Reply

Re: Week 3 Scenario 1
by Oyeombiliyetu Jason - Tuesday, 21 November 2023, 10:57 PM

1. Basal cell carcinoma, based on:

Continuous cutaneous trauma, by nose pricking

Race: White

Age: >40yo

Sex: Male

Usually single lesion, that is above the line drawn between the angle of the mouth and pinna

Well-margined (rolled), smooth pearly lesion with telangiectasis.

Slowly growing lesion (seen developing over months).

2. Differential diagnosis:

Squamous cell carcinoma: pearly appearance with telangiectasis, SCC is classically on the lower lip and BCC on the upper lip, histopathology shows, keratin pearls.

Merkel cell carcinoma: pearly appearance, rolled out borders with telangiectasis(rare), histopathology shows, clearer nuclei and frequent mitotic figures.

Amelanotic nodular melanoma: skin-coloured or pink lesion, histopathology shows, pleomorphic cells, with no melanin.

Others:

Clinical subtype	Differential diagnosis
Nodular	Intradermal nevus Squamous cell carcinoma Skin appendage tumors Seborrheic keratosis Dermatofibroma
Pigmented	Malignant melanoma Skin appendage tumor Compound nevus Blue nevus
Morpheaform	Scar Morphea Trichoepithelioma
Superficial	Bowen's disease Paget's disease Psoriasis Eczema
Fibroepithelioma	Skin tag Fibroma Papillomatous dermal nevus

Modified from Fitzpatrick's dermotology in general medicine. 8th ed. New York: McGrow-Hill, 2012:1294-1303[1].

3. Management in this patient

Considering, patient factors and preference, size, location, and Doctor experience, I would excise the lesion to get a confirmatory diagnosis, also to further prevent local involvement, especially with nasal cartilage. I would further have long-term follow-ups with patients to look out for recurrence.

4. Histological features

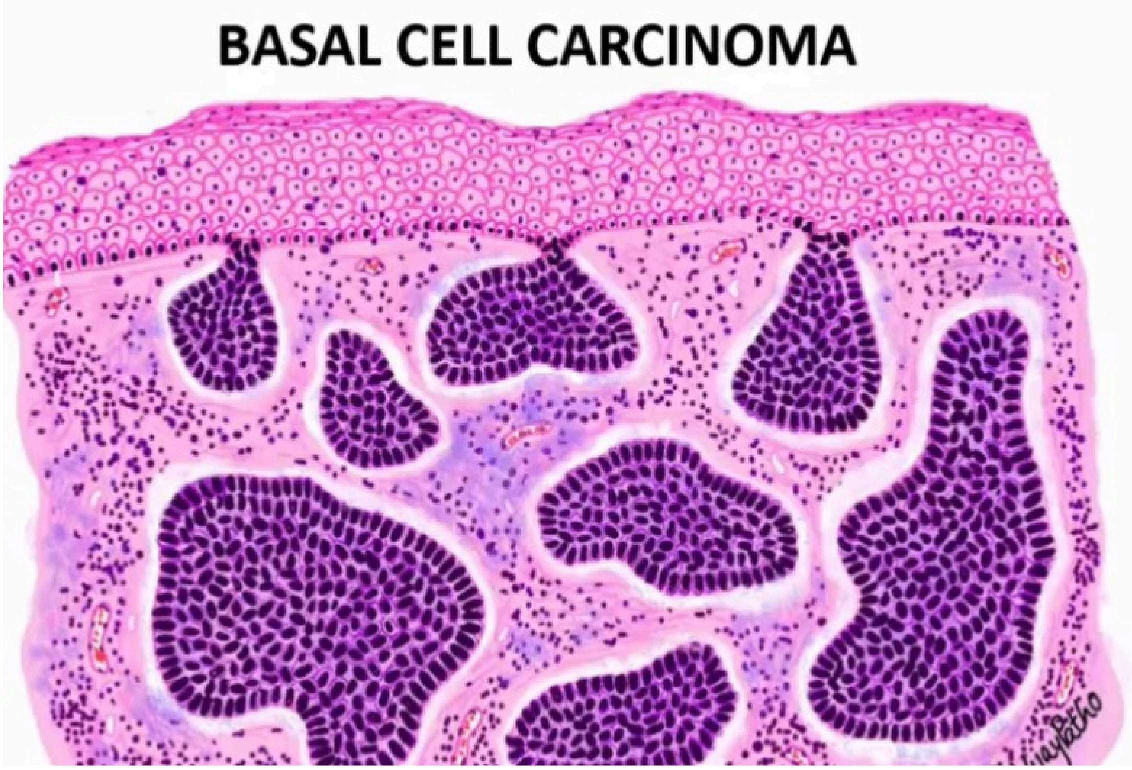
Basaloid tumours arising from the epidermis of follicular epithelium, and maintaining their connection to the epidermis

Nests of basaloid cells with scant cytoplasm and enlarged hyperchromatic nuclei

Peripheral palisading

Retracting artefact

Mixoid stroma degeneration



References:

1. Prof A. Oakley(2015). 'Basal cell carcinoma'. *Dermnet*. Available at: https://dermnetnz.org/topics/basal-cell-carcinoma. Accessed: 20 November 2023.

2. R. Bader(2022). 'Basal cell carcinoma'. *Medscape.* Available at: https://emedicine.medscape.com/article/276624-guidelines#g3?form=fpf&scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf. Accessed: 20 November 2023.

3. B. McDaniel(2022). 'Basal cell carcinoma'. *National Library of Medicine.* Available at: https://www.ncbi.nlm.nih.gov/books/NBK482439/. Accessed: 20 November 2023.

241 words

Reply

Re: Week 3 Scenario 1
by Sandeep Kumar - Wednesday, 22 November 2023, 2:58 PM

Good concise post
I wonder if we need to know histological features of these lesions as clinicians. It would be more appropriate question to histopatologists, what are your views on this.
Thanks

32 words

Reply

Re: Week 3 Scenario 1
by Sophie Holloran - Wednesday, 22 November 2023, 7:40 PM

Hi Sandeep,

I think that understanding some of the histological features can be really useful. One example where I think it is needed for Dermatologists is for Mohs surgery, as the dermatologists will assess the histology to see if the margins are clear in stages during the surgery. I think for dermatologists not doing Mohs it may be less useful in practical terms, but maybe more useful for knowledge base. I've not long started in dermatology so I am finding learning histology a steep learning curve!

Interested to know what others think too

93 words

Reply

Re: Week 3 Scenario 1
by Jillian Simpson - Thursday, 23 November 2023, 3:07 PM

Hi Sandeep

I have never heard of trigeminal trophic syndrome either.

I found it useful to read that it is an uncommon complication of damage to the trigeminal nerve. It can happen following trigeminal ablation by rhizotomy (Burge, Wallis & Martin, 2016). It can also be caused by damage to the trigeminal nerve or its central connection from cerebrovascular incidents like you said or by acoustic neuroma, syringobulbia, herpes zoster or trauma (Burge, Wallis & Martin, 2016).

The altered sensation, most commonly around the nose and nasal alar areas, then leads to repetitive, self-induced trauma, caused by picking or rubbing, which then leads to ulceration (Park & Koo, 2023).

Management is difficult and requires a multidisciplinary approach. Patient education and protecting the area with an occlusive dressing can be helpful, surgical repair with a skin flap has been recommended (Burge, Wallis & Martin, 2016).

References:
Burge, S.M., Wallis, D. and Matin, R. (2016). Oxford handbook of medical dermatology. Oxford: Oxford University Press. p549
Park, K.K., J. Koo.UpToDate. Skin Picking and excoriation Disorders. [Online] Available at: https://www.uptodate.com/contents/skin-picking-excoriation-disorder-and-related-disorders?search=trigeminal%20trophic%20syndrome&source=search_result&selectedTitle=2~3&usage_type=
[Accessed 23 Nov. 2023].

180 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:22 PM

Thank you Jillian, there is a bit more information here
Ho, C., Khan, S., Garza, I. (2023). Overview of craniofacial pain. Available at https://www.uptodate.com/contents/overview-of-craniofacial-pain?sectionName=Trigeminal%20trophic%20syndrome&search=Trigeminal%20trophic%20syndrome

24 words

Reply

Re: Week 3 Scenario 1
by Pooran Outar - Friday, 24 November 2023, 2:37 AM

Hello Sandeep,

In my opinion, it is crucial for future dermatologists to be well-versed in the histological features of skin lesions. Histopathology serves as the 'gold standard' for diagnosing both benign and malignant skin diseases. This method plays a pivotal role in aiding dermatologists in comprehending the overall patterns of skin diseases, “facilitating effective management, and allowing for a robust clinicopathological correlation” (Albasri and Ansari, 2019).

For instance, in the context of this week's case scenario involving a patient with BCC, histopathology reports prepared by pathologists are indispensable. These reports provide an assessment of the histological variants, contribute to risk determination for the recurrence of the illness (including staging and prognosis), and are instrumental in guiding the selection and evaluation of treatment options (Wu et al., 2014). The detailed histological information obtained through pathology reports is invaluable for tailoring a targeted and effective approach to patient care carried out by the dermatologist.

References:

- Albasri, A. and Ansari, I. (2019). The histopathological pattern of benign and non-neoplastic skin diseases at King Fahad Hospital, Madinah, Saudi Arabia. Saudi Medical Journal, 40(6), pp.548–554. doi:https://doi.org/10.15537/smj.2019.6.24205.
- Wu, A., Sun, M.T., Huilgol, S.C., Madge, S. and Selva, D. (2014). Histological subtypes of periocular basal cell carcinoma. Clinical & Experimental Ophthalmology, 42(7), pp.603–607. doi:https://doi.org/10.1111/ceo.12298.

209 words

Reply

Re: Week 3 Scenario 1
by Sandeep Kumar - Wednesday, 22 November 2023, 2:51 PM

Hi Derek,
Thank you for choosing this topic, Its very important clinically and we see a lot of patients with this presentation in general practice.

From the picture and history, it looks like a ‘barn door’ basal cell carcinoma (BCC).

Differentials to consider would be more aggressive kind of skin tumour - squamous, cell carcinoma, keratocanthoma, or benign lesions like seborrheic keratosis, dermatofibroma, trichoblastoma, viral lesions like molluscum contigiosum.

Treatment options.
Given the age of the patient patient 85 years old and an the background history of dementia I would discuss with the next of kin and carers about not treating this lesion as it is very slow growing, and a surgical treatment would leave a scar and potentially a non healing wound on the nose .

Given the size and site of this lesion, surgical excision would be the most appropriate treatment if the patient can tolerate it.
A more conservative curettage can also be considered.
In some cases, Radiotherapy or cryotherapy can be tried alone or along with topical treatments like 5FU and Imiquimod.

Histological features

As evident from the name, the cancer involves basal layer of the skin. Histologicaly we can find significant clusters of malignant basaloid cells in the centre and peripheral Palisading. There will also be mucoid stoma with spindles. In nodular BCC, we can also see amyloid deposits.

References

Paoli J, Gyllencreutz JD, Fougelberg J, Backman EJ, Modin M, Polesie S, Zaar O. Nonsurgical Options for the Treatment of Basal Cell Carcinoma. Dermatol Pract Concept. 2019 Apr 30;9(2):75-81. doi: 10.5826/dpc.0902a01. PMID: 31106008; PMCID: PMC6502306.

Trakatelli, M., Morton, C., Nagore, E., Ulrich, C., Del Marmol, V., Peris, K. and Basset-Seguin, N. (2014). Update of the European guidelines for basal cell carcinoma management. European Journal of Dermatology, 24(3), pp.312–329. doi:https://doi.org/10.1684/ejd.2014.2271.

El Ghissassi, F., Baan, R., Straif, K., Grosse, Y., Secretan, B., Bouvard, V., Benbrahim-Tallaa, L., Guha, N., Freeman, C., Galichet, L. and Cogliano, V. (2009). A review of human carcinogens—Part D: radiation. The Lancet Oncology, 10(8), pp.751–752. doi:https://doi.org/10.1016/s1470-2045(09)70213-x.

335 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:24 PM

Thank you Sandeep, good post.

5 words

Reply

Re: Week 3 Scenario 1
by Sadaf Basharat - Wednesday, 22 November 2023, 7:35 PM

Diagnosis:

The likely conclusion points to a basal cell carcinoma (BCC), considering aspects such as age, possibly fair skin, and the specific skin region affected. Additional backing for this diagnosis includes its gradual growth, tendency to bleed, and the observed features, like the 'rolled edge,' indicative of a nodular BCC commonly seen on the face.

Differentials:

```
The most important differentials for this patient are as follows:
```

1:Squamous cell carcinoma

2:Actinic keratosi

3:sebaceous Hyperplasia

Most appropriate management step in this patient:

The management strategy for BCC usually depends on its risk of recurrence. Nodular BCC are usually considered as low risk for recurrence.
For primary , nodular BCC, standard excision with 4-5mm margins and post operative margin assessment is usually used. MOHS is usually recommended for the excision of the lesion, which is performed by excising each layer carefully under a microscope (Oakley, 2015). But owing to the patient background of Dementia, we are not sure that this would be the best treatment option for this patient. So the treatment for this patient should be decided in a MDT.

```
(Aasi,2023)
```

```
What histological feature would support your diagnosis?
```

The most common histological features seen in BCC are nodules and/or strands of atypical basaloid cells that show nuclear palisading, cellular apoptosis and scattered mitotic activity in the dermis. Artifactual cleft formation may be seen between the tumor lobules and the surrounding stroma , which may be mucinous, solar elastosis, usually seen in dermis. `(Aasi,2023)`

References:

1. Oakley, A. (2015). *Basal cell carcinoma | DermNet NZ*. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/basal-cell-carcinoma.

2. Prieto-Granada, C. and Rodriguez-Waitkus, P., 2015. Basal cell carcinoma: epidemiology, clinical and histologic features, and basic science overview. *Current problems in cancer*, *39*(4), pp.198-205.

3. Aasi, S.(2023), Basal Cell Carcinoma / UpTodate. Available at : https://www.uptodate.com/contents/treatment-and-prognosis-of-basal-cell-carcinoma-at-low-risk-of-recurrence?search=basal%20cell%20carcinoma%20treatment&source=search_result&selectedTitle=1~150&usage_type=de

294 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:24 PM

Thank you Sadaf, good post.

5 words

Reply

Re: Week 3 Scenario 1
by Felica Mwinji Namukonde - Thursday, 23 November 2023, 7:19 PM

Nodular Basal Cell Carcinoma

based on patients age, gender, and fair skin presenting on face supports the diagnosis. usually appears as a flesh-colored or pink papule described as roll border peripherals which are more raised than the middle.
risk factors include;
ultra violent radiation (sun exposure) most common cause
use of tanning beds some studies show that 60% of users have an increased risk of developing it
Phototherapy; therapeutic exposure to psoralen and ultraviolent light for disorders like psoriasis increases the risk of NMSC.
personal history 40-50% people who have history of nodular basal cell carcinoma are at risk of recurrence
genetic factors

Important Differentials include;

1.Cutaneous Squamous Cell Carcinoma; usually arises from epidermal keratinocytes mostly appears in areas of photo damaged
skin.
2. keratocanthoma; keratolytic epithelial tumors resemble (SCC) appear as dome shaped or crateriform nodules with central keratotic core
3. Molluscum contagiosum, dermal nevi, sebaceous hyperplasia, small epidermal inclusion cyst (usually these are early nodular variants with small ulceration that can be identical to benign growths) (Wu A P 2023)

Management
treatment first-line (BCC)
Standard surgical excision margins 4 to 5mm are appropriate with postoperative margin close by use of skin grafts or flaps but closure is dependant on size, depth, and location of defect recurrence rate is less than 5% according to most trials.

Curettage and electrodissication this is best in older patients like in this case for patient that might have difficulties going bac for suture removal.

Second line
Imiquinoid 5% standard 5 times per week application is most appropriate for low risk nodular or superficial

lesions indicated if lesion is on low risk site, patient prefers to avoid surgery or if there is concern for not good cosmetic results.

References
Aasi S Z (2023) *Uptodate* Available at: https://www.uptodate.com/contents/treatment-and-prognosis-of-basal-cell-carcinoma-at-low-risk-of-recurrence?search=nodular%20basal%20cell%20carcinoma&source=search_result&selectedTitle=1~150&usage_type=defa
Accessed (23/11/23)

Petnel el at al (1995) *Surgery versus combined treatment with curettage and imiquimod for nodular basal cell carcinoma: One-year results of a noninferiority, randomized, controlled trial* Available at: *https://findit.southwales.ac.uk/discovery/fulldisplay?docid=cdi_proquest_miscellaneous_2438678511&context=PC&vid=44WHELF_USW:44WHELF_USW_NUI1&lang=*
Accessed (23/11/23)

326 words

Reply

Re: Week 3 Scenario 1
by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:24 PM

Thank you Felica, good post.

5 words

Reply

Re: Week 3 Scenario 1
by Mostafa Damghani - Friday, 24 November 2023, 9:19 PM

The Diangosis is Basal cell carcinoma

Differential Diagnosis

1- Squamous cell Carcinoma: tends to have a more hardened base on palpation.

2- keratoacanthomas: usually has a central keratin scale in the middle, and grows very rapidly.

3- Naevi: hair can grow from it, lacks rolled telangiectatic edge.

4- Actinic keratosis: lack ulceration, and blood vessel. feels rough and gritty. BCC tends to be more like a sore that fails to heal.

5- Sebacous hyperplasia: usually appears as a cluster of lesions rather than a single lesion. tends to have prominent telangectasia with yellowis lobules.

6- melanoma: rapidly growing black-grey discoloration with pigment spill

Management in this patient:
management for this patient, as this is a nodular BCC due to its ulceration and rolled over appearance of the edges would be Photodyamic Therapy, the use of topical imiquimod and 5-flurouracil should be reserved for superficial BCC. Surgery is preferably reserved for trunk and limbs and not for facial lesions due to risk of scarring, however it can be an option in well-defined nodular BCC.

Histological features of BCC

1- basaloid tumor arising from the epidermis in nests and islands with an elongated and more pgimented nuclei.

2- they show peripheral palisading and retraction

3- and empty spaces around the tumour islans called peritumoral clefting.

4- there is scanty cytoplasm as well with more prominent nuclei.

Refrences:

Oakley, A. (2015). *Basal cell carcinoma | DermNet NZ*. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/basal-cell-carcinoma.

Rook, A., Griffiths, C.E.M. and Al, E. (2016). *Rook's Textbook of dermatology*. West Sussex: Wiley Blackwell Basal cell carcinoma Chapter 141-1.

Habif, T.P. (2016). *Clinical dermatology a color guide to diagnosis and therapy*. St. Louis, Mo.] Elsevier Basal cell carcinoma page 801.

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16%

284 words

Reply

Re: Week 3 Scenario 1

by Derek Kayanja (Tutor) - Sunday, 26 November 2023, 12:26 PM

Thank you Mostafa, good post I suspect that you meant "diagnosis".

11 words

Reply

Re: Week 3 Scenario 1

by Mostafa Damghani - Sunday, 26 November 2023, 1:28 PM

Yes, apologies for the typo

Turnitin ID: 216892223

x

5 words

Reply

Re: Week 3 Scenario 1

by Bhavika Patel - Monday, 25 December 2023, 1:52 PM

The most likely diagnosis is non-pigmented Nodular Basal Cell Carcinoma which is the most common non-melanocytic skin cancer (NMSC).

There are rolled in edges, visible telangiectasia and slow-growth. It does have a typical appearance of a nodular BCC but a dermatoscopy image would be useful:

Pigmented BCC dermatoscopy features:

- Arborising vascular network and telangiectasia sometimes visible to the naked eye
- Leaf-life structures
- Multiple grey-blue globules
- Large blue-grey blotches
- Brown- grey specks of pigments
- Radial projections form a well circumscribed dark central point
- Foal ulcerations

Non-pigmented BCC dermatoscopy features:

- Flecks of grey colour
- Bluish-pink asymmetry
- Arborising vascular network and telangiectasia sometimes visible to the naked eye
- Focal ulceration
- Scaling and white areas of regression
- White irregular strands
- Disordered streaky chrystalline structures
- White rime around central ulceration
- Milia

Histology of BCCs:

- Epidermal basaloid tumour cells forming a central cleft next to the tumour stroma
- Central nuclei with surrounding scattered necrotic bodies
- Mucinous stroma
- Eosinophilic stroma without basaloid nests

Important differential diagnoses include the following:

- Nodular melanoma – most important to rule out
- Squamous cell carcinoma
- Melanocytic naevi
- Keratoacanthoma
- Trichiepilithelioma – fibromyxoid stroma, papillary mesenchymal bodies

Management for this patient would be ideally a Mohs surgery however given his vascular dementia it would be worth discussing with his Power of Attorney for Health or amongst a multidisciplinary team involving the elderly care and community nursing team to determine whether he would be suitable for surgery.

Tallon, B. Basal cell carcinoma pathology (2016). DermNet. Available at https://dermnetnz.org/topics/basal-cell-carcinoma-pathology [Accessed 19th December 2023]

270 words

Reply

◀ Day 2 Follow on Questions

Final Follow on Questions for week 3 ▶

◀ Complete Weekly Quiz

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Complete Weekly Quiz ▶